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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°166/MVE-BDBV/27/10/2026

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HIGHLIGHTS (last 24 hours)

As of October 27, 2026, 52 new confirmed cases and 32 deaths (20 community deaths and 12 in Ebola Treatment Centers) of Ebola Virus Disease (EVD) were reported in the provinces of Ituri (31), North Kivu (16), Tshopo (3), and Haut-Uélé (2). A total of 474 new samples were analyzed; 52 of these samples tested positive for Bundibugyo virus, representing a positivity rate of 11.0%.
No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10193	4960	48.7%	9 36	2,749	85.4%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

66 out of 178 health zones affected (37.1%)

Summary of key points from the last 24 hours

Fifty-two (52) **new confirmed cases** of Ebola virus disease (EVD) were reported in four provinces : **Ituri (31 cases), North Kivu (16 cases), Tshopo (3 cases), and Haut-Uélé (2 cases).** Among these cases, **20 community deaths** were reported **(12 in Ituri, 7 in North Kivu, and 1 in Haut-Uélé).** In addition, **12** confirmed deaths were recorded in Ebola Treatment Centers (7 in Ituri, 4 in North Kivu, and 1 in Haut-Uélé), bringing the total number of confirmed deaths reported during this period to **32 .**

Furthermore, 41 patients **(23 in Ituri, 17 in North Kivu and 1 in Haut-Uélé)** were declared recovered after obtaining two negative results in the Ebola virus disease (EVD) follow-up test.

As of October 27, 2026, the DRC had recorded **10,193 confirmed cases, including 4,960** deaths, representing a case fatality rate **of 48.7%.** Ituri province remained the epicenter, accounting for **71.4** % of reported confirmed cases and **59.6%** of new confirmed cases.

No new health zones have been affected in the last 24 hours.

Thus, 66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 27, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	31	7278	3,407	46.8%	28/36 (77.8%)
North Kivu	16	2361	1,325	56.1%	17/34 (50.0%)
Haut-Uélé	2	444	194	43.7%	7/13 (53.8%)
Tshopo	3	89	27	30.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	3	1	33.3%	1/11 (9.1%)
Total	**52**	**10,193**	**4,960**	**48.7%**	**66/178 (37.1%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 27, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7278**	**3407**	**46.8%**	**31**	**12**	**7**	**19**
Adja	11	1	9.1%				0
Aru	25	6	24.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	218	48	22.0%				0
Boga	2	2	100.0%				0
Bunia	1970	679	34.5%	12	8	3	11
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	38	12	31.6%				0
Komanda	193	118	61.1%				0
Lita	323	162	50.2%				0
Logo	14	9	64.3%				0
Lolwa	36	9	25.0%				0
Mahagi	5	1	20.0%				0

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Mambasa	88	42	47.7%	3	1	3	4
Mandima	456	242	53.1%	10	3	0	3
Mangala	394	172	43.7%	1	0	0	0
Mongbwalu	717	346	48.3%				0
Nia-Nia	339	197	58.1%				0
Nizi	842	306	36.3%	2	0	0	0
Nyankunde	139	43	30.9%	2	0	0	0
Rimba	22	6	27.3%				0
Rwampara	1188	424	35.7%	1	0	1	1
Tchomia	85	27	31.8%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2361**	**1325**	**56.1%**	**16**	**7**	**4**	**11**
Blessed	645	432	67.0%	2	1	3	4
Good	24	8	33.3%				0
Butembo	335	198	59.1%	2	1	0	1
Goma	1	0	0.0%				0
Kalunguta	94	41	43.6%	2	1	0	1
Katwa	667	409	61.3%	1	1	1	2
Kayna	3	2	66.7%				0
Kyondo	46	21	45.7%				0
Lubero	41	14	34.1%				0
Mabalako	106	38	35.8%	6	2	0	2
Alimbongo	5	3	60.0%				0
Manguredjipa	16	5	31.3%				0
Masereka	40	13	32.5%	1	0	0	0
Musienene	158	76	48.1%	1	1	0	1
Mutwanga	46	13	28.3%	1	0	0	0
Oicha	78	34	43.6%				0
Vuhovi	56	18	32.1%				0
Upper Uele	**444**	**194**	**43.7%**	**2**	**1**	**1**	**2**
Boma Mangbetu	46	19	41.3%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	112	52	46.4%	2	1	0	1
Pawa	110	63	57.3%				0
Rungu	6	3	50.0%				0
Wamba	166	55	33.1%	0	0	1	1
Tshopo	**89**	**27**	**30.3%**	3	0	0	0
Bafwasende	18	3	16.7%				0
Kabondo	20	2	10.0%	2	0	0	0
Lubunga	1	0	0.0%				0

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Makiso-Kisangani	34	14	41.2%	1	0	0	0
Mangobo	10	7	70.0%				0
Tshopo	5	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**3**	**1**	**33.3%**	0	0	0	0
Wapinda	3	1	33.3%				
Total	**10193**	**4960**	**48.7%**	**52**	**20**	**12**	**32**

***These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.**

NA: Not Applicable.

Table 3. Status of alerts notified by province for October 27, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	134	2	136	88	2	46	0	90	8
Bas Uélé	176	0	176	9	0	167	0	9	0
Ituri	1,467	72	1,539	164	71	1,208	1	225	124
North Kivu	1953	76	2029	215	73	1,567	3	288	186
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
North Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo	109	1	110	9	1	100	0	2	2
Grand Total	**3,839**	**151**	**3,990**	**485**	**147**	**3088**	**4**	**614**	**320**

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Figure 1. Evolution of cases by date of symptom onset over the last 21 days

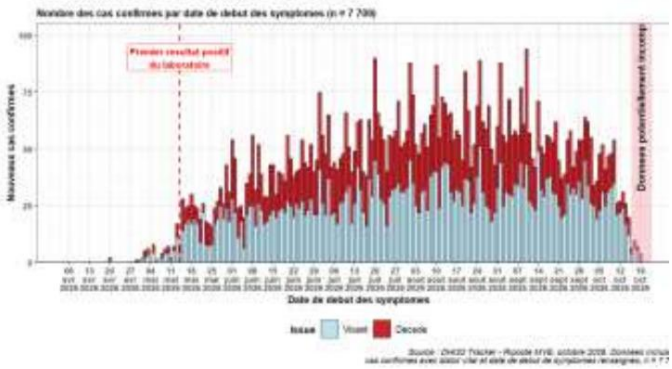
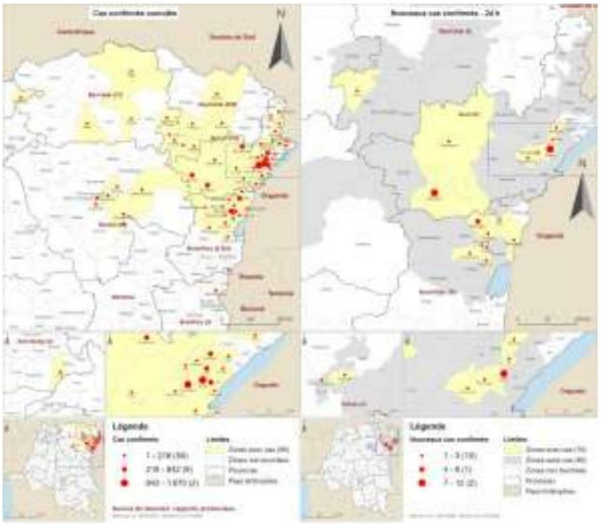


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

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As of October 27, 2026, **3,990 alerts** had been reported in the five provinces that submitted their data, including **3,839** alerts of living persons and **151 of deaths ; 3,724 (93.3%)** were verified, with 266 alerts remaining pending verification (171 in North Kivu and 95 in Ituri). Of the verified alerts, **632 (17.0%)** were validated as suspected cases and **3,092 (83.0%)** were invalidated. Of the 632 validated suspected cases, **614 (97.2%)**
320 were investigated **and** transferred to CT/CTE; 18 suspects remain uninvestigated (10 in Ituri and 8 in Tshopo) (Table 3).

As of October 27, 2026, **38,538** contacts were under follow-up in five provinces ; **32,930** were seen in the last 24 hours, representing a follow-up rate of **85.4%,** below the required threshold of **95.0%** but slightly higher than the previous day **(84.5%).** By province, the follow-up rate was 99.0% in Bas-Uélé (776/784), 91.5% in Ituri (14,766/16,136), 85.1% in Haut-Uélé (1,168/1,372), 82.6% in Nord-Kivu (15,626/18,911) and 44.5% in Tshopo (594/1,335).

2.1.2. Update on checkpoint and point of entry (PoE/PoC) activities

Main actions

On October 27, 2026, ten (10) alerts were notified to the PoE/PoC: 5 in Ituri, 2 in South Kivu, 2 in Bas-Uélé, and 1 in Tshopo; no alerts were reported in North Kivu or Haut-Uélé. In Ituri, the 5 live alerts (3 at the Bela PoC, Mandima Health Zone, 1 at the Mudzi Bala PoC, Bunia Health Zone, and 1 at the Pluto PoC, Mongbwalu Health Zone) were validated and investigated; 2 were transferred to the CTE (40%; 2/5). In South Kivu, 2 bodies repatriated from Burundi (one from a stroke and the other from a traffic accident) were intercepted at the Kavimvira PoE (Uvira Health Zone) and secured (100%; 2/2). In Tshopo, the alert from PoC PK15 on the Banalia road (7-year-old child) was validated and investigated. In Bas-Uélé, the two alerts from PoC PK2 (Poko health zone) were invalidated after verification. In total, six active alerts were validated, three of which were transferred (50%), and all were investigated within 24 hours (100%).

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Table 4. Status of key PoE/PoC indicators by province, as of October 27, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	93.5% (58/62)	95% (20/21)	56% (5/9)	100% (18/18)	67% (8/12)	45% (10/22)	83% (119/144)
Strategic PoC/PoE	1	0	0	0	0	0	1
Number of past travelers	157,408,953	8,220	7 072 22	661 18 493		2,008,303	464
Proportion of travelers screened	97%	97%	99%	99%	99%	98%	97%
% of travelers who washed their hands	97%	97%	99%	98%	99%	99%	97%
99.7 % of travelers made aware		97%	100%	100%	100%	89%	99%
Number of alerts notified	05	0	02	01	0	02	10
Number of validated live alerts	05	0	0	01	0	0	6
Number of lifeless bodies intercepted	0	0	02	0	0	0	2
% of suspected cases transferred to CT/CTE	40% (2/5)	0%	0%	100% (1/1)	0%	0%	50% (3/6)
% of lifeless bodies swabed/secured	0%	0%	100% (2/2)	0%	0%	0%	100% (2/2)
% of suspected cases investigated within 24 hours	100% (5/5)	0%	100%	100% (1/1)	0%	0%	100% (6/6)

2.1.3 Challenges

In Ituri, 3 of 5 suspects validated at the PoCs of Mandima and Nizi have not been transferred (0 ambulances available, refusal), the Avakubi PoC remains non-functional due to insecurity.

In North Kivu, the Butuhe PoC remains closed since the security incident of September 26, 2026; **in Bas-Uélé and South Kivu,** the completeness of reports remains low (45.5% and 56%).

2.2 Laboratory

2.2.1 Main actions

- **Ituri:** 31 new positive results (19 living and 12 dead) out of 238 new samples analyzed (positivity rate: 13.0%)
- **North Kivu:** 16 new positive results (9 living and 7 dead) out of 213 new samples analyzed in Beni and Butembo (positivity: 7.5%).
- **Tshopo:** 3 new positive results (3 live) out of 6 new samples analyzed (positivity: 50.0%).
- **Haut-Uélé: 2** new positive results (1 living and 1 dead) out of 13 new samples analyzed (positivity: 15.4%).
- **Bas Uélé: 4 samples received and analyzed (4 live) the results were negative.**

2.3. Infection Prevention and Control (IPC/EDS)

2.3.1. Main actions

- **Ituri :** 28 rings open out of 34 (82.4%), 53 households decontaminated out of 66 (80.3%) and 18 FOSA out of 18 (100%); 44 EDS carried out out of 72 alerts, including 23 postponements (61.1%), and 58 bodies swabed.
- **North Kivu :** 33 out of 37 rings open (89.2%), 22 out of 38 households decontaminated (57.9%), and 25 out of 25 ESS (100%); 52 EDS carried out out of 89 alerts, and 57 bodies swabed.

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• **Tshopo:** 1 ring opened in Kabondo with decontamination of an ESS (REKAPI) and a household around the 82nd and 83rd cases; 16 ESS followed in 4 ZS and 138 providers briefed; 1 EDS carried out and 1 body swabé out of 10 alerts.

• **Haut-Uélé:** 3 out of 4 rings open (75.0%), 7 out of 7 households decontaminated (100%) and 4 out of 4 ESS (100%), as well as 5 classrooms and 10 toilets in Isiro; 3 EDS carried out on 3 alerts and 3 bodies swabés.

• **Bas-Uélé:** no ring opened nor decontamination carried out; 19 service providers and 7 hygienists were briefed in Ganga, 17 service providers and 4 hygienists in Viadana, where 4 priority ESS were provided with PCI-WASH inputs; no EDS alert.

2.3.2. Challenges.

• **In North Kivu,** household decontamination rates were low (57.9%) in Katwa, Butembo, and Beni; no households received kits (0/22); 3 swab failures (Lubero and Mutwanga); and 37 DHS (Disinfection Health Services) were postponed. **In Ituri,** IPC (Infection Prevention and Control) teams were insufficient in Mangala, Mandima, Damas, and Mambasa; IPC supplies were insufficient in Mandima and Niania; and 16 DHS failures (22%) were reported. **In Haut-Uélé,** there was resistance to household decontamination in Ibambi State (Pawa), and IPC supervisors in Pawa refused to work due to unpaid bills. **In Tshopo,** 9 out of 10 bodies were not swabs. **In Bas-Uélé,** 16 out of 20 priority health facilities lacked functional triage, no priority health facilities had mattresses, and there were no hygiene kits for households around cases.

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2.4. Holistic care

2.4.1. Main actions

• **In Ituri,** 63 new admissions (all suspected cases) were recorded, compared to 99 discharges, including 23 recoveries and 7 confirmed deaths. Thus, 359 patients (121 confirmed and 238 suspected) are hospitalized, representing an occupancy rate of 34%; 336 out of 340 patients (98.8%) received hot meals, and all children on LNPE/SLM were treated (19/19 at the CTE).

In North Kivu, 120 new admissions (13 confirmed and 107 suspected) were recorded, compared to 130 discharges, including 17 recoveries and 4 confirmed deaths. Thus , 523 patients (178 confirmed and 345 suspected) are currently receiving care, 160 of whom are outside of Ebola Treatment Centers (ETCs), for an occupancy rate of 88.5% (363/410) for ETCs.

• **In Tshopo,** 7 new admissions (3 confirmed and 4 suspected) were recorded; 22 patients (12 confirmed and 10 suspected) are hospitalized at the CTE Météo (occupancy rate: 61.1%; 22/36) and all received hot meals.

• **In Haut-Uélé,** 9 new admissions (4 confirmed and 5 suspected) against 9 discharges, including 1 recovered; 32 patients (16 confirmed and 16 suspected) hospitalized for 149 beds, representing an occupancy rate of 21.5%.

2.4.2. Challenges

In Ituri, the amount of blood received remains insufficient to meet the needs. Furthermore, patients have
37 patients received food and are in critical condition. **In North Kivu,** Ebola treatment centers (ETCs) are nearing capacity (88.5%) and 160 patients are receiving treatment outside of ETCs; in Tshopo, 2 confirmed cases escaped in Bafwasende; in Haut-Uélé, network coverage remains poor and nutritional supplies are lacking

2.5. Vaccination

2.5.1. Main actions

In Ituri, vaccination of people living with HIV (PLHIV) using Ervebo (MSF support) and research on its effectiveness against BBDV-EVI are continuing, bringing the number of people vaccinated to 1,873 (766 women and 1,107 men), as are the Partners, EBO-PEP, and Oracle trials. **In Bas-Uélé,** vaccination has begun in the Dakwa health zone, and compassionate vaccination continues in Buta; 1,785 people have been vaccinated out of a target of 13,202 (13.5%).

2.5.2. Challenges

• **In Bas-Uélé,** the start-up of the UCCs is not effective and coverage remains very low in Bili (0 out of 1,067), in Aketi (7.8%) and in Titule (7.8%); in Haut-Uélé, the vaccination of PPLs is stopped due to the failure of the generator set of the PEV antenna in Isiro.

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2.6. Continuity of Care

2.6.1. Main actions • **In North Kivu,** the

CSSE evaluation of CH Rapha and CH Maman Muponyaji (Oicha) noted the absence of triage and emergency services, with only 5 out of 24 providers (20.8%) and 5 out of 25 (20.0%) trained in IPC; in Tshopo, the evaluations of CS Anoalite (Mangobo) and CM Geson (Tshopo) noted a non-effective triage area and only 4 out of 9 agents (44.4%) vaccinated at CM Geson; in Haut-Uélé, the LLIN distribution campaign is postponed to November 4, 2026.

2.6.2. Challenges

Low reporting of data from the pillar (no data from Ituri, Bas-Uélé and Sud-Ubangi); lack of running water, oxygen and blood products at CH Rapha (North Kivu); assessed recovery plans for ESS not implemented in Tshopo; 28 days of shortage of BCG and 0.05 ml syringes at the PEV antenna in Isiro (Haut-Uélé).

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **In Ituri,** 29 out of 38 feedbacks (76.3%) and 10 out of 13 incidents (76.9%) were resolved within 48 hours and 315 community alerts, including 73 deaths, were notified; the CREC facilitated the pre-listing of 1,073 contacts around 20 confirmed cases, the decontamination of 18 out of 18 households, the completion of 19 out of 21 EDS, the announcement of 20 out of 20 positive results and the transfer of 55 out of 57 suspects (96.5%); 16,576 home visits reached 59,633 people and the training of 181 actors, including 163 RECOs, was launched in Nizi.

- **In North Kivu,** 11,946 home visits reached 48,792 people and 387 educational talks reached 7,384 people; 364 out of 650 alerts (56.0%) were notified, 15 out of 22 feedbacks (68.2%) were resolved within 48 hours and 1,674 out of 4,750 people (35.2%) were engaged in community actions; none of the 3 incidents were resolved.

- **In Bas-Uélé,** 3 home visits reached 14 people and 3 mass awareness campaigns reached 321 people in the Viadana health zone, where CREC also supported 3 PCI interventions; no community alerts were notified.

- **In Tshopo,** 3,537 home visits and 12 mass awareness campaigns reached more than 110,000 people and the 52 community alerts notified were resolved (Kabondo and Makiso-Kisangani).

In Haut **-Uélé,** 312 home visits, 86 educational talks, and 5 mass awareness campaigns in schools reached 3,724 people; 10 community alerts (Boma Mangbetu and Wamba) and 3 death alerts were reported, and 5 out of 5 feedback issues were resolved.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- **In Ituri,** 30 out of 36 confirmed cases (83.3%), 90 out of 107 confirmed cases and 110 out of 175 suspected cases under follow-up, as well as 44 out of 63 new suspected cases, have been taken care of, 13 out of 15 separated and orphaned households (86.7%) have been supported, 103 children are being followed up, 9 out of 9 children are in daycare (2 admissions), 8 out of 23 have been reintegrated into the community, 24 out of 24 have been followed up in the community, 305 caregivers and 89 PPLs have been supported and 49 results have been announced, including 30 positive ones.
- **In North Kivu,** 30 of the last 36 confirmed cases (83.3%) and 202 new suspects were taken care of, 37 out of 40 (92.5%) separated and orphaned households were supported, 13 recovered were reintegrated and 101 results were announced, including 36 positive ones.

- **In Tshopo,** all confirmed (1 new and 9 under follow-up) and suspected (3 new and 10 under follow-up) cases were taken care of, 1 out of 2 resistances were overcome and 783 people benefited from community psychoeducation.

- **In Haut-Uélé,** 3 new confirmed cases, 6 confirmed and 7 suspected cases under follow-up as well as 4 new suspected cases were taken care of, 9 results announced (including 3 positive) and 22 accompanying persons and 46 PPL supported; in Bas-Uélé, 410 people (258 men and 152 women) participated in psychoeducation sessions.

2.8.2. Challenges

In Ituri, only 15 out of 28 affected health zones (53.6%) are covered, and coverage remains incomplete around the most recent cases in Mangala (4/6) and Bunia (7/9). In North Kivu, coverage around the most recent cases remains low.

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In 9 and 10 affected health zones, there are no nurseries. **In Tshopo,** the APS (Auxiliary Police Officers) in the Wanie-Rukula health zone are not trained, and the pillar lacks communication units and means of transport.

2.9. Logistics

2.9.1. Main actions

In North **Kivu,** MAPI kit supplies have been delivered to the PEV branch in Butembo, and the delivery of supplies to Walikale (via Nyaleke) is ongoing; in Tshopo, construction of the Bafwasende CTE and the expansion of the CTE are underway.

The weather continues and 150 handwashing kits have been prepared for the EPSP (Primary, Secondary and Professional Education), but 115 motorcycles are still needed for the ZS (Special Zones). **In Bas-Uélé, security** construction ...

2.9.2. Main actions

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In Tshopo, the health workers' strike and the attacks and threats against the response teams have an impact on surveillance activities and contact tracing.

2.10. PSEA (Prevention of Sexual Exploitation and Abuse)

2.10.1. Main actions

In Ituri, 83 staff (42 men and 41 women) were briefed and signed the code of conduct in Bunia and Rimba, and 53 people were sensitized in the community; **in North Kivu,** 28 response providers (14 women and 14 men) were briefed in Butembo and signed the code of conduct, and 1,099 people (486 men and 613 women) were sensitized in the community; **in Tshopo,** 60 participants from the Cokis, Neema and Umoja health centers were sensitized on the PEAHS, the referral circuit and zero tolerance.

2.10.2. Challenges

• Coverage of affected areas remains low: in Ituri, only 15 out of 36 health zones (41.7%) have access to the reporting mechanism and only 2 partners (WHO and IOM) report their PSEA activities; in North Kivu, only 4 out of 34 health zones (11.8%) know the reporting mechanism; in Tshopo, collection tools are lacking and the community complaint management mechanism is not installed in all health zones of Kisangani; only 3 out of 8 health districts (37.5%) reported their PSEA activities in the last 24 hours.

KEY MESSAGES

In the last 24 hours, 52 new confirmed cases have been reported in four provinces, including 31 in Ituri, 16 in North Kivu, 3 in the Tshopo and 2 in Haut-Uélé. Transmission remains particularly marked in the health zones of Bunia (12 cases), Mandima (10 cases) and Mabalako (6 cases). Of the 32 confirmed deaths reported, 20 are community deaths (12 in Ituri, 7 in North Kivu and 1 in Haut-Uele). The contact tracing rate stands at 85.4% (32,930/38,538), below the 95% threshold, with the rates the lowest in Tshopo (44.5%), where the health workers' strike and Attacks against teams disrupt monitoring, and in North Kivu (82.6 %). The situation underlines the need to strengthen early detection, the contact tracing (particularly in Tshopo), dignified burials and secured and the transfer of suspects, in particular to the PoC of Bela (Mandima) due to the lack of an ambulance.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP
Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd **Incident Manager:** Professor Dr. AHUKA MUNDEKE Steve Email: steve.ahuka@unikin.ac.cd

Operations Manager
Dr. KOMBE NGWAMA John

Editorial Team
Professor Dr. MULANGU KATULUMBA Félix-Médard
Dr. Jules Katusi Biselenge
Mrs. MAYELE AFUA Marie
Dr. DIUR MANANASHI Floryan
Mr. KAMUINA KEBELA John

Contact:
www.insp.cd
contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd